

Wisconsin Department of Health Services

STATEMENT OF DEFICIENCIES AND PLAN OF CORRECTION		(X1) PROVIDER/SUPPLIER/CLIA IDENTIFICATION NUMBER: 0015448	(X2) MULTIPLE CONSTRUCTION A. BUILDING: _____ B. WING: _____	(X3) DATE SURVEY COMPLETED R 02/25/2026
NAME OF PROVIDER OR SUPPLIER ALICE & LOUISES ADULT FAMILY HOME		STREET ADDRESS, CITY, STATE, ZIP CODE 814 E 6TH ST MARSHFIELD, WI 54449		
(X4) ID PREFIX TAG	SUMMARY STATEMENT OF DEFICIENCIES (EACH DEFICIENCY MUST BE PRECEDED BY FULL REGULATORY OR LSC IDENTIFYING INFORMATION)	ID PREFIX TAG	PROVIDER'S PLAN OF CORRECTION (EACH CORRECTIVE ACTION SHOULD BE CROSS-REFERENCED TO THE APPROPRIATE DEFICIENCY)	(X5) COMPLETE DATE
{M 000}	INITIAL COMMENTS On 02/25/2026, Surveyor conducted a verification visit at Alice and Louises Adult Family Home. Two (2) of 4 deficiencies from Statement of Deficiency (SOD) #EXDO11 were corrected. Two (2) new deficiencies were identified. Four (4) deficiencies were identified in total. Under statutory provisions of WI Stat. Ch. 50 a \$200 revisit fee is being assessed. Census: 4	{M 000}		
{M 234}	88.04(2)(g)2 COMMUNICABLE DISEASE The licensee shall ensure that no one who has a communicable disease reportable under ch. HSS 145 may work in a position in the adult family home where the disease would present a significant risk to the health or safety of the residents. This Rule is not met as evidenced by: Based on record review and interview, the provider did not ensure Caregiver E was screened for illness detrimental to residents by a physician, a registered nurse or a physician's assistant, within 90 days before the start of providing services. This is a repeat deficiency. See Statement of Deficiency (SOD) EXDO11, dated 02/07/2025. Findings include: On 02/25/2026 at 10:42 A.M., Surveyor reviewed Caregiver E's employee record. Caregiver E was	{M 234}		

LABORATORY DIRECTOR'S OR PROVIDER/SUPPLIER REPRESENTATIVE'S SIGNATURE

TITLE

(X6) DATE

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{M 234}	Continued From page 1 hired on 10/09/2025. Caregiver E's employee record did not contain evidence of a communicable disease screening. On 02/25/2026 at 11:25 A.M., Owner F stated that s/he was unaware of the need to have staff screened for illness detrimental to residents prior to providing services. S/he acknowledged that Caregiver E did not have one completed and inquired if screening staff for a communicable disease was a new requirement. On 02/25/2026 at 11:38 A.M., Surveyor reviewed and provided Owner F with a document titled, "F-01679 COMMUNICABLE DISEASE/TUBERCULOSIS SCREENING QUESTIONNAIRE."	{M 234}		
M 276	88.05(3)(a) Homelike Environment An adult family home shall be safe, clean and well-maintained and shall provide a homelike environment. This Rule is not met as evidenced by: Based on observation and interview, the provider did not ensure the home was clean and well-maintained. Findings include: This facility is licensed to serve up to 4 residents	M 276		

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M 276	Continued From page 2 in the client groups advanced aged, irreversible dementia/Alzheimer's, physically disabled, developmentally disabled, emotionally disturbed/mental illness. On 02/25/2026 between 11:00 AM and 1:11 PM, Surveyor observed the following: -- Thick dust accumulation located on the ceiling fan in the kitchen. Four residents were observed sitting at the table directly under the ceiling fan eating lunch. -- Dark, thick, dust-like substance covering the entire top surface of the hood fan located above the stove. Directly under the hood fan, Surveyor noted a pot of water boiling on the stovetop that was utilized to cook lunch for the residents. On 02/25/2026 at 1:11 PM, Surveyor interviewed House Manager G and inquired about the dust on the hood fan and ceiling fan. S/he acknowledged the dust on the fans and said, "We have tried everything" to clean the dust on the hood fan but was unsure when it was last completed. Surveyor took a damp napkin and successfully scrubbed away a small section of dust from the hood fan. House Manager G observed Surveyor remove a section of dust from the hood fan and stated, "We could probably get a scrub brush and get rid of that."	M 276			
M 458	88.07(3)(a) PRESCRIPTION MEDICATIONS Every prescription medication shall be securely stored, shall remain in its original container as received from the pharmacy and be stored as specified by the pharmacist.	M 458			

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M 458	Continued From page 3 This Rule is not met as evidenced by: Based on observation and interview, the provider did not ensure that every prescription medication remained securely stored. Findings include: This facility is licensed to serve up to 4 residents in the client groups advanced aged, irreversible dementia/Alzheimer's, physically disabled, developmentally disabled, emotionally disturbed/mental illness. On 02/25/2026 at 9:35 AM, Caregiver E led Surveyor from the entryway to the kitchen. Upon entering the kitchen, Surveyor observed multiple bottles containing medication on the kitchen countertop located to the right of the sink. Surveyor noted a number of bottles displayed a pharmacy label. On 02/25/2026 at 9:58 AM, Caregiver E left the kitchen to assist a resident. Surveyor remained in the kitchen and was unable to see Caregiver E. At approximately 10:04 AM, Surveyor observed another resident walk through the kitchen. On 02/25/2026 at 11:44 AM, Surveyor requested to view where medication is securely stored. Owner F acquired a key from a kitchen drawer	M 458			

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M 458	Continued From page 4 and unlocked the upper kitchen cabinet to the right of the sink. Owner F acknowledged that the key was stored in an unlocked kitchen drawer, was not secured, and stated, "It is usually on the staff's wrist," and gestured to the wristband on the keyring. On 02/25/2026 at 11:50 AM, Surveyor reviewed the following medications on the kitchen countertop with Owner F: - Two bottles of "Benzoyl Peroxide Topical Wash 10%". Both bottles contained medication and one bottle had a piece of tape on it with a handwritten label which read, "USE Daily in AM." One bottle displayed a yellow sticker which read in part, "DATE OPENED ...EXP DATE ...INITIAL." The handwritten items on the label were illegible and appeared to have rubbed off. - One bottle of "Carboxymethylcellulose Sodium 0.05% Eye Drops," with an expiration date of 03/05/2025. Surveyor noted there was medication remaining in the bottle. - Two bottles of "Betamethasone Dipropionate Lotion USP, 0.05%." One bottles expiration date was 05/16/2024, and the other bottles expiration date was illegible. - One bottle of "Nystatin Topical Powder" with an expiration date of 09/03/2025. Surveyor noted there was medication remaining in the bottle. - One bottle of "Clindamycin Phosphate Topical Lotion 1%" with an expiration date of 09/30/2024. Surveyor noted there was medication remaining in the bottle. On 02/25/2026 at 11:51 AM, Owner F and House Manager G acknowledged the above medications were currently being stored on the kitchen counter. House Manager G stated that the medications were expired, not currently being	M 458		

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M 458	Continued From page 5 used, but should have been securely stored until disposal. Both Owner F and House Manager G ultimately acknowledged that the medications were not securely stored and therefore accessible to anyone in the home.	M 458		
{M 570}	88.10(3)(I) Safe Physical Environment Individuals, except for correctional residents, have basic rights which they do not lose when they enter an adult family home. A resident shall have a safe environment in which to live. The adult family home shall safeguard residents who cannot fully guard themselves from environmental hazards to which they are likely to be exposed, including conditions which would be hazardous to anyone and conditions which would be or are hazardous to a particular resident because of the resident's condition or disability. This Rule is not met as evidenced by: Based on observation, interview, and record review, the provider did not ensure 4 of 4 residents were safeguarded from environmental hazards when the home's water temperature exceeded 120 degrees F at 2 of 2 faucets tested. This is a repeat deficiency. See Statement of Deficiency (SOD) EXDO11, dated 02/07/2025. Findings include: This facility is licensed to serve up to 4 residents in the client groups advanced aged, irreversible dementia/Alzheimer's, physically disabled, developmentally disabled, emotionally	{M 570}		

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{M 570}	Continued From page 6 disturbed/mental illness. On 02/25/2026 at 10:20 AM and 10:32 AM, Surveyor checked water temperatures in the facility at 2 faucets used daily by residents and staff. The water temperature at the kitchen sink registered at 125.9 degrees F. The water temperature at the bathroom faucet registered at 128.0 degrees F. On 02/25/2026 at 11:10 AM, Surveyor observed a sheet of paper hanging on the refrigerator titled "Fridge/Freezer/Water Temp Check". The document contained a column which prompted daily fridge, freezer, and water temperature checks during February 2026. The document included daily checks for the fridge and freezer during the month of February. The document did not include any water temperature checks during the month of February 2026 and there was a line drawn through the water temperature column. Surveyor asked Caregiver E whether water temperature testing is completed. S/he responded, "Probably not, it is supposed to be done daily." On 02/25/2026 at 11:40 AM, Surveyor discussed the above concerns with Owner F. Owner F asked Caregiver A for the facility thermometer. Caregiver A responded, "We have a meat one ...I bought one," and gestured to the thermometer Owner F was holding. Owner F attempted to take the water temperature at the kitchen sink with the facility's thermometer (a culinary thermometer used to test meat). Surveyor observed steam rising from the pouring water. On 02/25/2026 at 11:42 AM, Surveyor tested the	{M 570}		

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{M 570}	Continued From page 7 water temperature in the kitchen sink at 135 degrees F. Owner F verified the reading and stated, "Tell the residents not to use the water right now as we adjust." "Exposure to hot water is a potential environmental danger for residents. Elderly residents and residents with mental and physical disabilities may have neurological conditions that prevent instant recoil from hot water. Because these individuals do not instantly react to dangerous temperature levels, they are at particular risk for injury. Hot water can cause scalding; e.g., second and third degree burns in which the skin blisters and swells. In such instances, skin does not return to normal and forms scar tissue upon healing. These burns may lead to permanent disability. Second and third degree hot water burns can occur at the following rates at the following temperatures: 110 degrees F, 13 minutes 120 degrees F, 10 minutes 127 degrees F, 1 minute 130 degrees F, 30 seconds 140 degrees F, 6 seconds." (Source: Publication DHS P-01942 - 08/2017). The provider did not ensure 4 of 4 residents were safeguarded from environmental hazards when the water temperature exceeded 120 degrees F at 2 of 2 faucets in the home.	{M 570}		